Size: 165x120 mm

165.00 mm

07/11/2023

120.00 mm

Iron (III) Hydroxide Polymaltose Complex

HAEMO XL CAPSULE

COMPOSITION:
Each hard gelatin capsule contains:
Iron (III) Hydroxide Polymaltose Complex
Eq. to elemental Iron 100mg

PHARMACODYNAMICS:
The polynuclear iron (III)-hydroxide cores are superficially surrounded by a number of non-covalently bound polymaltose molecules resulting in an overall complex molecular mass (Mw) of approximately 50 kD, which is so large that diffusion through the membrane of mucosa is about 40 times smaller than that of the hexaqua-iron(II) units.
The complex is stable and does not release ionic iron under physiological conditions. The iron in the poly-nuclear cores is bound in a similar structure as in the case of physiologically occurring ferritin. Due to this similarity, only the iron (III) of the complex is absorbed by an active absorption process. By means of competitive ligand exchange, any iron binding protein in the gastro-intestinal fluid and on the surface of the epithelium, take up iron (III). The absorbed iron is stored mainly in the liver, where it is bound to ferritin. Later in the bone marrow, it is incorporated into haemoglobin.
Iron (III)-Hydroxide Polymaltose Complex has no pro-oxidative properties such as there are in iron II) salts. The susceptibility of lipoproteins such as Very Low Density Lipoprotein (VLDL) + Low Density Lipoprotein (LDL) to oxidation is reduced. Iron (III) Hydroxide Polymaltose Complex does not cause teeth staining.

PHARMACOKINETICS:
Studies using the twin-isotope technique (55Fe and 59Fe) show that absorption of iron measured as haemoglobin in erythrocytes is inversely proportional to the dose given (the higher the dose, the lower the absorption). There is a statistically negative correlation between the extent of iron deficiency and the amount of iron absorbed (the higher the
iron deficiency, the better the absorption). The highest absorption of iron is in the duodenum and jejunum. Iron which is not absorbed is excreted via the faeces. Excretion via the exfoliation of the epithelial cells of the gastro-intestinal tract and the skin as well as perspiration, bile and urine only amount to approximately 1 mg of iron per day. For women, iron loss due to menstruation has also to be taken into account

Pharmacode

INDICATION:
In the treatment of anaemia due to iron deficiency. Treatment and prophylactic therapy of iron deficiency during pregnancy. This product should only be used in pregnancy after the first thirteen weeks.

DOSAGE AND ADMINISTRATION
Adults: 1 to 2 capsules daily depending upon the severity of anaemia. Medical advice should be sought if symptoms do not improve after four weeks of use of this product as these symptoms may reflect an underlying disease process.

WARNINGS & PRECAUTIONS:
Jauhkandaripadacapaiankanak-kanak.
Keep out of reach of children.
Please consult your pharmacist/doctor before taking this product.
Contents irritating to mucosa, to ask the doctor.
1. The response to iron therapy should be regularly monitored.
2. The additional requirements for folic acid should be borne in mind when treatment with iron is carried out during pregnancy.
3. In cases of anaemia due to infection or malignancy, the substituted iron is stored in the reticulo-endothelial system, from which it is mobilised and utilised only after curing the primary disease.
4. Caution is advised in individuals with a family history of haemochromatosis or an iron overload syndrome.It should be noted that these conditions may be under diagnosed.
5. Overdose may be fatal.

CONTRAINDICATION:
- Known hypersensitivity to iron polymaltose or to any of the excipients.
- Iron overload (e.g. haemochromatosis, haemosiderosis)
- Disturbances in iron utilisation (e.g. lead anaemia, sidero-achresticanaemia, thalassaemia)
- Anaemia not caused by iron deficiency (e.g. haemolyticanaemia or megaloblastic anaemia due to vitamin B12 deficiency)

PREGNANCY AND LACTATION:
This product should only be used in pregnancy after the first thirteen weeks.
Pregnancy Category A
Reproduction studies in animals did not show any foetal risk. Controlled studies in pregnant women after the first trimester have not shown any undesirable effects on mother and neonates. There is no evidence of a risk during the first trimester and a negative influence on the foetus is unlikely.

LACTATION:
Breast milk naturally contains iron bound to lactoferrin. It is not known how much iron from the complex is passed into breast milk. The administration of capsules is unlikely to cause undesirable effects to the nursed child.
During pregnancy and lactation capsule should be used only after consulting a physician.

SIDE EFFECTS:
Very rarely gastro-intestinal discomfort, vomiting, constipation or diarrhoea can occur.
A dark colouration of the stool is of no clinical significance.

OVERDOSE:
In cases of overdosage neither intoxication nor iron overload have been reported to date because the iron from the active substance Ferric-Hydroxide-Polymaltose Complex is not present in the gastro-intestinal tract as free iron and is not taken up by the organism by passive diffusion.

Pharmacode

DRUG INTERACTIONS:
Until now interactions have not been observed. Since the iron is complex-bound, ionic interaction with food components (phytin, oxalates, tannin etc) and concomitant administration of medicaments (tetracyclines, antacids) are unlikely to occur.
The haemoccult test (selective for Hb) for the detection of occult blood is not impaired and therefore there is no need to interrupt iron therapy.

PRODUCT DESCRIPTION
Maroon opaque coloured cap and Maroon opaque coloured body, size '1' on-brittle hard gelatin capsule containing brown coloured granular powder

STORAGE:
Store at temperatures not exceeding 30°C.

AVAILABILITY:
PVDC/PVC/Al blister pack 10's (Box of 3 x10's).

DATE OF REVISION: 07.11.2023

Manufactured by:
XL LABORATORIES PRIVATE LIMITED
E-1223, Phase-I Extn. (Ghatal), RIICO
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